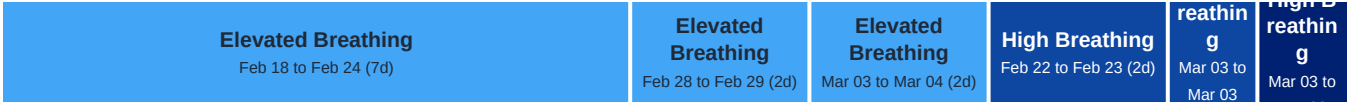


Patient ID: SallenPeriod: February 6 to March 18, 2024Report Date: March 19, 2024Coverage: 676/993h (68%)

Patient's monitoring period was less than 90 days; this report covers all available data. Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Patient clinical status over 42 days from Feb 06 to Mar 18

■ HR ■ Breathing



Last 24 Hours

Last 24 Hours: NORMAL

30-Day Tier: GREEN

Vital	Avg	Min	Max
Heart Rate	59 bpm	45 bpm	84 bpm
Breathing	17 brpm	9 brpm	48 brpm

Limited data in the final 24h (16/24h); values reflect available readings.

Last 24 hours: one elevated breathing episode in the evening, otherwise within normal range.

Episodic Events (last 24h)

Date	Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
Mar 17	8 PM to 9 PM	1h	1	Elevated Breathing	25	12	45	Check SpO2 and lung sounds; assess for fluid overload or early infection

23 episodic events Detected, spanning 24h total. Conditions: Elevated Breathing, High Breathing, Very High Breathing. Priority grade: Low

Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage
23	24h	Elevated Breathing	<1	68%

Major Findings: Stable baseline

Date	Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
Mar 03	4 PM to 5 PM	1h	1	Very High Breathing	41	28	44	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
Feb 22	3 PM to 4 PM	2h	2	High Breathing	31	22	43	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
Mar 03	10 PM to 11 PM	1h	1	High Breathing	31	13	45	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
Feb 18	7 PM to 8 PM	8h	8	Elevated Breathing	27	9	57	Check SpO2 and lung sounds; assess for fluid overload or early infection
Feb 28	10 PM to 11 PM	6h	5	Elevated Breathing	27	9	54	Check SpO2 and lung sounds; assess for fluid overload or early infection
Mar 03	3 PM to 4 PM	2h	2	Elevated Breathing	26	16	43	Check SpO2 and lung sounds; assess for fluid overload or early infection

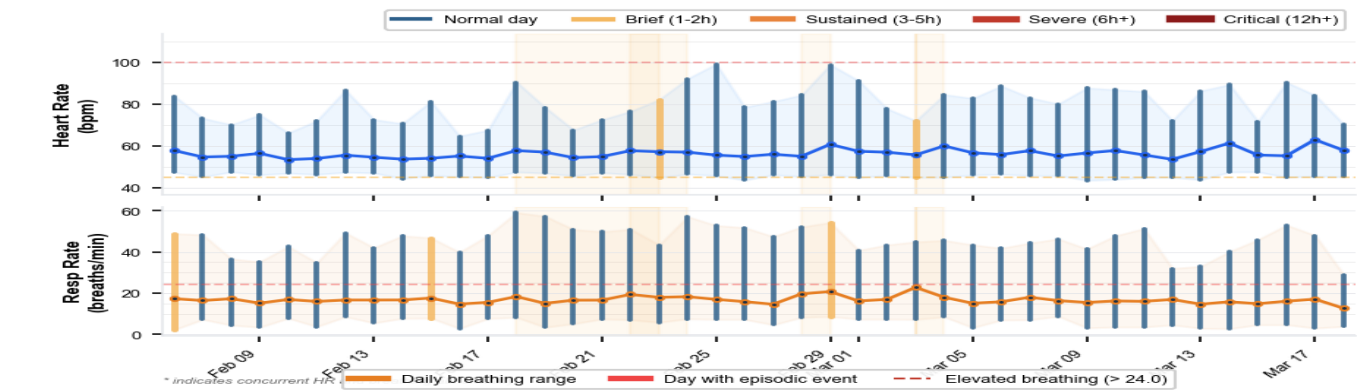
RR values outside physiologic range (6–60 breaths/min).

Suggested Clinical Review Actions

- Elevated Breathing (avg 27 brpm, peak 57 brpm): Check SpO2 and lung sounds; assess for fluid overload or early infection
- High Breathing (avg 31 brpm, peak 43 brpm): Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
- Very High Breathing (avg 41 brpm, peak 44 brpm): Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
- Overall trajectory shows 6 unstable phases with 19 bpm variability. Consider increasing monitoring frequency and lowering escalation threshold.

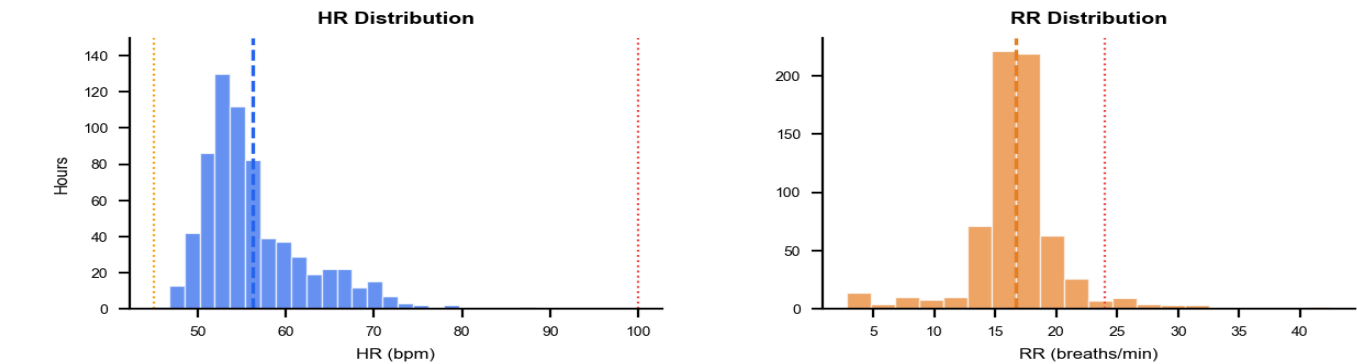
Clinical Pattern Observations:

- 1. Breathing variability: RR spread 11 brpm (10 to 21).
- 2. Clustered pattern: Episodic events on 5 of 42 days (11%).

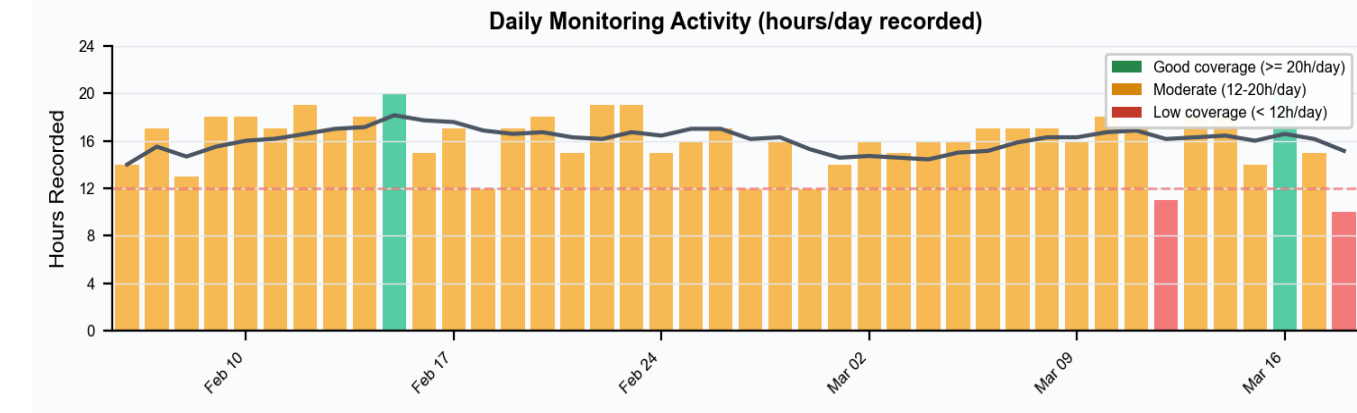


Red bars indicate days with detected episodic events.

Vital Sign Distribution



Monitoring Activity



Bars indicate monitoring hours/day. Red threshold line indicates clinical monitoring baseline.

■ HR episode ■ RR episode ■ Recorded, no episode ■ No data

Clinical Alerting Thresholds

- Very Low HR < 40 bpm
- Low HR < 45 bpm
- Elevated HR > 95 bpm
- High HR > 100 bpm
- Very High HR > 110 bpm
- Elevated Breathing > 24 brpm
- High Breathing > 30 brpm
- Very High Breathing > 40 brpm

Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.